NCCP 00586 · VERSION 3

Your treatment, explained

Locally advanced rectal cancer with concurrent radiotherapy · NCCP 00586 v3 · Document revised 4 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Capecitabine + rectal radiotherapy

7-day cycle | Source reconciled; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Stop capecitabine and contact oncology for significant diarrhoea, painful mouth or hand-foot symptoms affecting activity, vomiting that prevents drinking, fever or sudden illness. Do not restart after a toxicity interruption until the team advises.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Open your CUH contact / treatment card

Sources: hse · patient

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

- How each drug works
- How the treatment fits together
- Your 7-day timeline
- Baseline and cycle-specific checks
- Whole treatment, scans and checks
- Chemotherapy and radiotherapy timing
- Tablets, visits and practical planning
- Your treatment aim and duration
- Taking capecitabine safely
- Your radiotherapy visits
- Tests and checks
- Recognise symptoms and act
- Daily life and other medicines
- Review, recovery and follow-up
- Supportive medicines
- Understanding the evidence
- Questions to ask
- Sources

UNDERSTAND YOUR TREATMENT

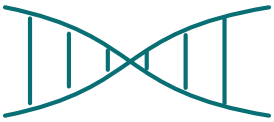
How the treatment works

Capecitabine tablets help pelvic radiotherapy work against locally advanced rectal cancer. Your team explains how this course fits with surgery and any other chemotherapy.

ORAL CHEMOTHERAPY

Capecitabine

Xeloda



Becomes fluorouracil in the body

After you swallow capecitabine, your body changes it into fluorouracil. This interferes with the DNA and RNA processes cells use to grow. Normal cells can also be affected, which helps explain mouth soreness, bowel symptoms and changes in blood counts.

Swallowed tablet → Converted to fluorouracil → Cell growth disrupted

What to watch for

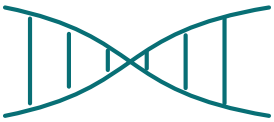
Stop tablets and contact the team for significant diarrhoea, painful mouth ulcers, hand-foot soreness affecting activities, fever or dehydration. Chest pain needs urgent assessment. Do not restart until instructed.

Sources: nccp · patient · smpc

LOCAL RADIATION TREATMENT

Pelvic radiotherapy

External beam treatment



Targets the pelvic treatment area

Radiation is directed at the planned area. Capecitabine helps sensitise cancer cells to it. Nearby bowel, bladder and skin may also become irritated. Effects can build over the weeks and may continue after the final session.

Planned radiation field → Damage to cancer-cell DNA → Cells lose the ability to multiply

What to watch for

Tell the radiotherapy team about diarrhoea, pain, skin soreness or urinary changes. Discuss possible longer-term bowel, bladder, sexual and fertility effects before treatment.

Sources: radiation · rectal

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Explore your existing anatomy and side-effect map

REGIMEN-SPECIFIC SIDE EFFECTS

Your anatomical side-effect map

A picture of side effects

Match each symptom box to its numbered body location. Tell your team about new symptoms.

1 Mouth

Capecitabine

Painful ulcers or reduced intake: stop tablets and call.

2 Chest

Capecitabine

New chest pain: stop tablets and seek urgent assessment.

3 Bowel / stoma

Capecitabine + radiotherapy

Significant diarrhoea, increased output or dehydration needs prompt advice.

4 Hands / feet

Capecitabine

Pain or peeling affecting daily activity: stop tablets and call.

5 Blood / infection

Capecitabine

Fever, shivers or sudden illness: contact oncology immediately.

6 Hydration / urine

Capecitabine safety

Reduced urine, dizziness or inability to drink needs assessment.

7 Pelvic skin

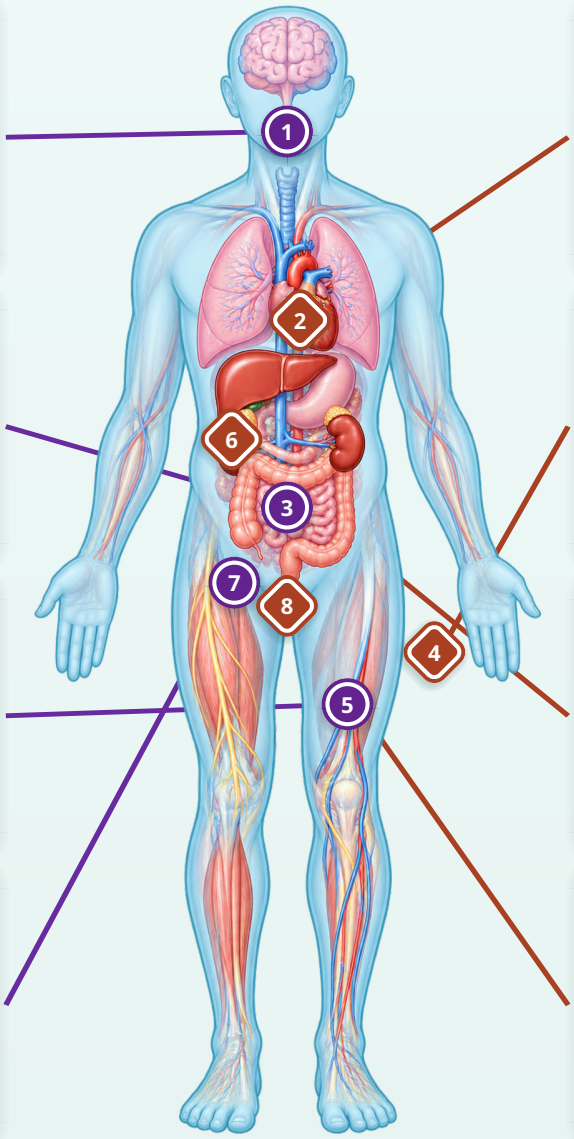
Radiotherapy

Report painful, broken or weeping skin in the treatment area.

8 Bladder / sexual health

Radiotherapy

Report urinary pain and discuss fertility and sexual effects.



Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

THE BIGGER TREATMENT PLAN

How the treatment fits together

TABLETS + RADIOTHERAPY

Weekly chemoradiation

D1 D5 D7

Capecitabine twice daily on Days 1–5 of each 7-day cycle, alongside radiotherapy, for up to 5–6 cycles. Follow the written plan for treatment days and interruptions.

The treating team confirms the aim, duration and next steps. The visual pattern does not replace your prescription.

Sources: nccp

THE WHOLE TREATMENT COURSE

Chemotherapy and radiotherapy: given together

Capecitabine tablets and radiotherapy run alongside each other during this course. You do not need to enter any dates.

Course →	Week 1	Week 2	Week 3	Week 4	Week 5	Week 6*
Chemotherapy tablets	Capecitabine: Days 1–5 each week					If prescribed
Radiotherapy	Radiotherapy appointments during the same weeks					If planned

Inside each prescribed week

D1	D2	D3	D4	D5	D6	D7
Tablets	Tablets	Tablets	Tablets	Tablets	No tablets	No tablets

Capecitabine is taken twice daily on Days 1–5. The weekly tablet break is different from finishing the whole course. Radiation appointments follow the centre's plan. If a session changes or either treatment is paused, contact the team for the revised tablet instructions.

*The NCCP course is a maximum of 5–6 weekly cycles. The sixth week is shown as optional. Your radiation team confirms the actual number of sessions and any bank-holiday changes.

> **After the course: recovery and reassessment. Surgery or other chemotherapy, if planned, follows your individual multidisciplinary plan; no universal interval is implied.**

Educational reference, not a personal appointment calendar. NCCP 00586 v3, page 1

BASELINE → TREATMENT → FOLLOW-UP

Treatment checks across the course

Relative stages, not equally spaced dates. Agree who books each check, reviews results and gives advice if a check is missed.

Check	Before starting	First treatment	During treatment	Phase / dose review	After treatment
Bloods and symptoms	DPD, blood count, kidney/liver; agree RT and tablet plan	Confirm tablet days and meal instructions	Weekly/pre-cycle bloods; bowel, skin, urine and intake review	Do not restart or make up tablets without team advice	Radiation/surgical review and management of persistent effects
Treatment review	Confirm exact cancer setting and biomarkers	Explain expected benefit and how it will be assessed	Report new symptoms between visits	Review response, tolerability and active medicines	Agree the next treatment or surveillance plan
Tablets and radiation	Coordinate prescription, radiation bookings and transport	Twice-daily tablets on Days 1–5	No routine tablets on Days 6–7; maximum 5–6 cycles	Coordinate any tablet or radiation interruption	Completion does not mean all side effects end immediately

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00586 v3 · official regimen

THE WHOLE TREATMENT JOURNEY

Capecitabine + rectal radiotherapy: treatment and checks

Locally advanced rectal cancer with concurrent radiotherapy

Take capecitabine twice daily on Days 1–5 of each 7-day cycle with radiotherapy, for a maximum of 5–6 cycles. Days 6–7 are not routine tablet days. Follow the team's written instructions if radiotherapy or tablets are interrupted.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

On a small screen, swipe the map sideways. The checks below give the same timing in words.

Treatment, tests and reviews across the course

Care / check	Before starting	First treatment	During treatment	Phase / dose review	After treatment
Bloods and symptoms NCCP 00586	DPD, blood count, kidney/liver; agree RT and tablet plan	Confirm tablet days and meal instructions	Weekly/pre-cycle bloods; bowel, skin, urine and intake review	Do not restart or make up tablets without team advice	Radiation/surgical review and management of persistent effects
Treatment review NCCP 00586	Confirm exact cancer setting and biomarkers	Explain expected benefit and how it will be assessed	Report new symptoms between visits	Review response, tolerability and active medicines	Agree the next treatment or surveillance plan

Care / check	Before starting	First treatment	During treatment	Phase / dose review	After treatment
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What each check is for

1

Confirm your personal plan

Before starting

Capecitabine tablets help pelvic radiotherapy work against locally advanced rectal cancer. Your team explains how this course fits with surgery and any other chemotherapy.

Ask / act: Agree medicines, treatment days, duration and monitoring.

Sources: NCCP 00586

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

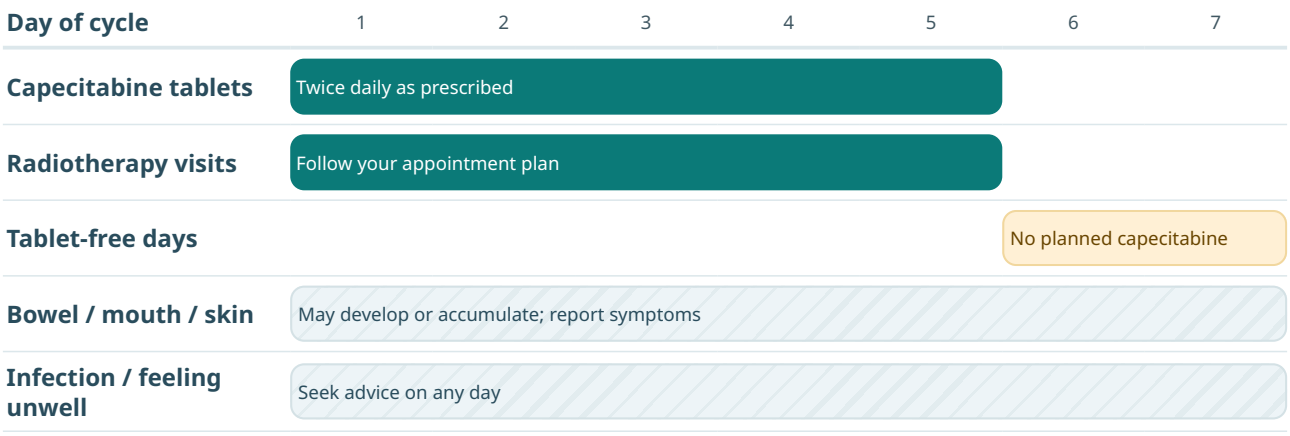
This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-04 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.32

Day 1 → Day 7: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Take capecitabine twice daily on Days 1–5 of each 7-day cycle with radiotherapy, for a maximum of 5–6 cycles. Days 6–7 are not routine tablet days. Follow the team's written instructions if radiotherapy or tablets are interrupted.



Day of cycle	1	2	3	4	5	6	7
Pelvic skin / bladder	Can build over the treatment weeks						

Your written plan comes first: Capecitabine twice daily on Days 1–5 of each 7-day cycle, alongside radiotherapy, for up to 5–6 cycles. Follow the written plan for treatment days and interruptions.

Act when symptoms occur: Do not wait for the next visit or use the chart to decide a symptom is safe.

Symptoms can build: Bowel, mouth and hand-foot symptoms may accumulate and can continue during tablet-free days.

Across the course: Radiotherapy symptoms can build over successive weeks and continue after treatment finishes.

> **Why these bands stay broad:** Treatment bands show the NCCP schedule. Symptom bands are broad teaching illustrations: they do not represent measured daily risks or predict when you will feel well. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00586 v3 · official regimen ↗ · eviQ: taking capecitabine and managing symptoms (NCCP defines the Irish schedule) ↗ · NCI: radiation side effects ↗

> **There are no guaranteed "safe days".** Act on fever, sudden illness or a new serious symptom whenever it occurs. Do not wait for the next appointment.

Read the whole cycle as text

Day 1–5: Treatment days

Capecitabine twice daily on Days 1–5 of each 7-day cycle, alongside radiotherapy, for up to 5–6 cycles. Follow the written plan for treatment days and interruptions. Use the current tablet label. Swallow whole with water with or within 30 minutes of food; do not crush or cut tablets.

Day 6–7: Planned tablet-free days

Do not take leftover tablets to replace doses missed earlier. Side effects may continue during this interval. Keep the oncology contact plan available and confirm the next review before restarting.

PRACTICAL TREATMENT GUIDE

Tablets, visits and practical planning

A simple guide to what happens before treatment, on treatment days and at home. Your own service confirms appointments and prescriptions.

STEP 1

Treatment phases and appointments

Take capecitabine twice daily on Days 1–5 of each 7-day cycle with radiotherapy, for a maximum of 5–6 cycles. Days 6–7 are not routine tablet days. Follow the team's written instructions if radiotherapy or tablets are interrupted.

STEP 2

Checks after treatment

Keep reviewing bowel, bladder, skin, fatigue and nutrition effects during and after the course. Late effects and surgical assessment follow the radiation/colorectal team plan. Do not automatically extend tablets to make up missed radiation days or continue beyond the planned course.

STEP 3

Access and practical support

No IV port or infusion pump is needed for capecitabine itself; separate treatments may differ. Radiation appointments, tablet supply, meals and transport should be planned together.

Keeping tablets safe

Keep the original labelled packaging away from children. Follow the storage label and pharmacy handling instructions. Do not share tablets or discard them in household waste.

If treatment is interrupted

Tell the team which doses were missed and why. Never extend the course, double a dose or move tablet days yourself. Ask for a revised written plan.

Questions about your appointments

Ask about transport, blood-test location, pharmacy collection and a named contact for appointment changes. Booking arrangements differ between centres.

Radiotherapy planning and recovery

The radiation team plans the field and explains positioning and preparation. Treatment-related bowel, bladder and skin effects can continue after the last session; keep reporting them.

Open practical treatment guide · Printable practical guide

Sources: NCCP 00586 v3 · official regimen · eviQ: taking capecitabine and managing symptoms (NCCP defines the Irish schedule) · NCI: radiation side effects. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 Concurrent capecitabine and pelvic radiotherapy

No automatic high/moderate IV antiemetic course for capecitabine. Written tablet-day and missed/held-dose instructions; do not double doses or extend to weekends without the team's plan. Diarrhoea/dehydration, hand-foot and pelvic skin care; nutrition and urgent-contact support.

Sources: nccp · patient · smpc

TREATMENT REFERENCE · 01

Your treatment aim and duration

The same medicine can be used for different purposes.

Why this treatment?

Capecitabine tablets are given with pelvic radiotherapy for locally advanced rectal cancer. The tablets help the radiation treatment work; your team confirms how this fits with surgery and other treatment.

Sources: nccp · patient

How long?

The NCCP weekly schedule runs for up to 5–6 cycles with radiotherapy. The radiation oncologist specifies the dose, field and visits; the MDT confirms later surgery or other therapy.

Sources: nccp · patient

Alternatives and decisions

Discuss the expected benefit, burdens, reasonable alternatives and the option of supportive care. The best choice depends on your cancer, fitness and preferences.

Sources: nccp · patient

TREATMENT REFERENCE · 02

Taking capecitabine safely

Tablets are active chemotherapy, with a specific schedule.

Meals and doses

Use your current prescription. Swallow tablets whole with water with or within 30 minutes after food. Do not cut, chew or crush them; ask pharmacy if swallowing is difficult.

Sources: nccp · patient

Missed doses or vomiting

Do not replace a missed or vomited dose. Never double the next dose. Contact the team for repeated vomiting or uncertainty about the schedule.

Sources: nccp · patient

Pause and restart

Stop tablets for significant diarrhoea, painful mouth/hand-foot symptoms affecting activity, fever or dehydration and seek advice. A toxicity-related pause requires instructions before restart.

Sources: nccp · patient

Different tablet strengths

Different strengths may be combined to make your dose. A reduction changes tablet numbers: use the new label and do not calculate a change from this page.

Sources: nccp · patient

TREATMENT REFERENCE · 03

Your radiotherapy visits

The radiation team gives a separate, individual plan.

Before treatment

Planning appointments define the area to treat and how you will be positioned. Follow the exact bowel and bladder preparation instructions from your centre.

Sources: rectal · radiation

Daily sessions and tablets

NCCP 00586 specifies capecitabine Days 1–5 of each week. Confirm the plan if a session is missed, postponed or falls around a bank holiday; do not add weekend tablets yourself.

Sources: nccp

After the course

Side effects can accumulate and continue after treatment. Ask about follow-up and possible late bowel, bladder, sexual or fertility effects.

Sources: radiation

TREATMENT REFERENCE · 04

Tests and checks

Monitoring helps the team decide whether treatment can continue.

DPD testing

DPD helps break down fluoropyrimidine medicines. Low activity can cause severe toxicity; testing before treatment helps guide the plan but cannot prevent every serious reaction.

Sources: nccp · patient

Blood, kidney and liver results

Counts, kidney and liver function are checked before each cycle. Diarrhoea and poor intake can worsen kidney function and make tablets harder to tolerate.

Sources: nccp · patient

Your symptoms matter

Report bowel frequency, stoma output, mouth soreness, hand-foot pain, chest symptoms and what daily activities you can no longer manage.

Sources: nccp · patient

Additional checks

Radiotherapy reviews also address pelvic skin, bowel, urinary and nutritional problems.

Sources: nccp · patient

TREATMENT REFERENCE · 05

Recognise symptoms and act

Do not wait for the next appointment if you feel unwell.

Bowel and hydration

Report diarrhoea or increased stoma output early. Inability to drink, dizziness, very little urine, severe pain or blood in stool needs prompt assessment.

Sources: nccp · patient

Hands, feet and mouth

Pain, cracking, peeling or ulcers that affect normal activities or eating/drinking need advice. Gentle preventive care helps, but does not replace reporting.

Sources: nccp · patient

Chest symptoms and infection

Stop capecitabine and seek urgent assessment for chest pain. Fever, shaking chills or sudden illness needs immediate oncology contact, even if the temperature is normal.

Sources: nccp · patient

Treatment-specific effects

Pelvic radiation can cause bowel, bladder and skin irritation, with possible later effects. Ask about sexual health and fertility before treatment.

Sources: nccp · patient

TREATMENT REFERENCE · 06

Daily life and other medicines

Make the plan fit your practical needs.

Eating, drinking and activity

Aim for tolerable meals and fluids; contact the team if intake falls. Gentle activity and rest can be adjusted to your energy. Ask for dietetic help if needed.

Sources: nccp · patient

Medicine and supplement checks

Check new medicines with oncology pharmacy. Warfarin can require closer INR checks; phenytoin levels may need monitoring. Brivudine is an important prohibited interaction.

Sources: nccp · patient

Family, work and fertility

Ask about infection exposure, work, intimacy, contraception, fertility and any caring responsibilities. Use the medicine-specific duration of precautions given by your team.

Sources: nccp · patient

Procedures and travel

Plan transport and support around appointments. Ask which symptoms or blood-count problems affect travel and when to contact the service.

Sources: nccp · patient

TREATMENT REFERENCE · 07

Review, recovery and follow-up

Your plan can change as treatment progresses.

Dose changes are individual

A pause or reduction may be needed because of symptoms, blood results or kidney function. The team considers the treatment aim and other options.

Sources: nccp · patient

After the last dose

Keep reporting new or persistent symptoms after treatment ends. Your team confirms follow-up, scans and recovery support; the timeline is not a forecast.

Sources: nccp · patient

Prepare for review

Bring questions, symptoms and missed-dose records. Tell the team what matters most to you and ask them to explain the next decision.

Sources: nccp · patient

Why this treatment is offered

Capecitabine was an effective oral alternative to infused fluorouracil when combined with radiotherapy in the study. Outcomes included the whole treatment programme, including surgery.

Capecitabine was an effective oral alternative to infused fluorouracil when combined with radiotherapy in the study. Outcomes included the whole treatment programme, including surgery.

Sources: trial-v08421

PREPARE FOR THE CONVERSATION

Questions worth asking

What is the aim of my treatment?

Capecitabine tablets are given with pelvic radiotherapy for locally advanced rectal cancer. The tablets help the radiation treatment work; your team confirms how this fits with surgery and other treatment.

Which days do I take the tablets?

Capecitabine twice daily on Days 1–5 of each 7-day cycle, alongside radiotherapy, for up to 5–6 cycles. Follow the written plan for treatment days and interruptions.

What if I forget a dose or vomit?

Do not take an extra dose. Take the next planned dose at its usual time unless treatment has been stopped; report repeated vomiting and check with the team if unsure.

When should I stop tablets and call?

Stop capecitabine and contact oncology for significant diarrhoea, painful mouth or hand-foot symptoms affecting activity, vomiting that prevents drinking, fever or sudden illness. Do not restart after a toxicity interruption until the team advises.

Why do I need blood tests if I take tablets at home?

Blood counts, kidney and liver results, together with symptoms, guide whether to continue and at what dose.

Do I need a port or a pump?

No take-home chemotherapy pump is used. Capecitabine is swallowed, so a central line is not required just to give it.

What happens after treatment?

Ask for your review and follow-up plan. Continue reporting symptoms after the last dose; further therapy depends on the treatment aim and response.

Which checks continue when treatment changes or finishes?

Keep reviewing bowel, bladder, skin, fatigue and nutrition effects during and after the course. Late effects and surgical assessment follow the radiation/colorectal team plan. Do not automatically extend tablets to make up missed radiation days or continue beyond the planned course.

> **Try explaining the plan back:** Which days do you take tablets? What do you do if you miss a dose? Which symptoms mean stop tablets and call? Who do you contact out of hours?

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00586 v3. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00586 v3 · official regimen
- eviQ: taking capecitabine and managing symptoms (NCCP defines the Irish schedule)
- Capecitabine product information: medicines, interactions and precautions
- HSE: chemotherapy side effects and urgent advice
- NCI: radiation side effects
- NCI: rectal cancer treatment
- Hofheinz rectal chemoradiotherapy trial: primary trial report
- NSABP R-04
- NCCP emetogenic classification V6
- NCCP NCIS Medical Oncology antiemetics V8

SACTCheck · Developed by Dr Paul O'Brien · Sources checked 2026-11-04.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

Written source addresses

https://healthservice.hse.ie/documents/6396/586_v3_Capecitabine_825_and_RT.pdf

https://www.eviq.org.au/medical-oncology/colorectal/rectal/84-rectal-locally-advanced-capecitabine-chemoradia/patient-information

https://www.cancer.gov/about-cancer/treatment/types/radiation-therapy/side-effects

https://www.medicines.org.uk/emc/product/14545/smpc

https://www2.hse.ie/conditions/chemotherapy/side-effects/

https://www.cancer.gov/types/colorectal/patient/rectal-treatment-pdq

https://pubmed.ncbi.nlm.nih.gov/22503032/

https://pmc.ncbi.nlm.nih.gov/articles/PMC4849360/

https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_SACT_Induced_Nausea__pWgDCrm.pdf

https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf